A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Kashif Ahmad
Kialla Medical Clinic

Dear Dr Ahmad,

Re: Mrs Batool Zehra
DOB: 04/12/1989 Phone: 0456209494
Address: 4 Mcewen Street, SHEPPARTON, VIC, 3630
Email: sbatoolzehra786@gmail.com
Medicare: 3608379822 / 2

Summary

- **Management of refractory gastro-oesophageal reflux symptoms, likely functional in nature, and burning mouth syndrome.**
- **Dose reduction and potential cessation of amitriptyline due to medication-induced somnolence.**
- **Trial of Creon for suspected fat malabsorption and weight loss.**
- **Escalation of rabeprazole to twice-daily dosing.**
- **No action required by you.**

I had the pleasure of conducting a review with Batool Zehra. She continues to experience bothersome gastrointestinal symptoms, primarily related to reflux and a sensation consistent with burning mouth syndrome. She has identified various dietary triggers, particularly high-fat foods such as ghee, butter, and cream. While she suspected potatoes and okra were causative, we discussed the likelihood that these symptoms are instead triggered by the inclusion of onion or garlic. Furthermore, Batool has noted weight loss, currently weighing 50 kg, and expresses a desire to return to her previous weight of 56 kg.

Regarding her current pharmacotherapy, Batool reports significant daytime drowsiness and somnolence secondary to amitriptyline, despite taking the medication early in the evening. We discussed the role of amitriptyline in modulating her visceral hypersensitivity and managing her burning mouth symptoms; however, the side-effect profile is currently limiting its utility. Her last gastroscopy, performed in September last year, was unremarkable, which provides reassurance against significant structural pathology.

Impression and Plan

My impression is that Batool's symptoms are likely driven by visceral hypersensitivity.

I have recommended increasing her rabeprazole to 20 mg twice daily, taken before breakfast and dinner. To address her fat/oil intolerance and assist with weight gain, I have provided an empiric prescription for Creon (pancreatin). She is to take this lipase supplement with any fatty or fried meals. I also suggested the over-the-counter enzyme Lactese for use with dairy products.

To manage the somnolence associated with amitriptyline, I have suggested she attempt a reduced dose of 5 mg (half a tablet) daily. If the drowsiness persists even at this dose, she will wean off the medication by taking

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it every second day for one week before cessation. I have cautioned her that the cessation of amitriptyline may result in a recurrence of her reflux and abdominbal pain - for which she hasn't presented to ED in some time.

If there is no symptomatic improvement within the next six weeks, I have advised Batool to book a repeat gastroscopy. Should her symptoms remain refractory thereafter, we will consider oesophageal manometry and a 24-hour pH study to further characterise her condition. I have scheduled a follow-up review in three months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires

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Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065